



UAT4_2

Date: 9/28/2025

Case: UAT4_2

Pages: 241

Page(s)	Testimony
p.1:1	15 Deposition of Richard Sackler, M.D., commenced; witness sworn in by the notary public.
p.1:16	25 Q: Who were the principal owners of the document? A: The medical and regulatory departments; FDA had determinative power.
p.2:1	10 Q: Turn to page 5, paragraph 34. A: Discusses labeling of withdrawal symptoms and the importance of over-reporting.
p.2:11	20 Q: What does the analysis conclude? A: On June 26, 2000, Purdue supervisors sent the full text of an osteoarthritis study article.
p.5:1	15 Q: "Were all speakers vetted?" A: "Everyone should have been vetted... no excuse for not validating their degrees and licenses."
p.5:16	25 Q: "How were they vetted?" A: "I don't know precisely, but they definitely should have all been vetted."
p.6:1	8 Q: "Have companies been criticized for incentivizing prescriptions?" A: "Yes, pharmaceutical and medical device companies have faced such criticism."
p.7:9	22 A: "Patients often took Tylenol every 4 hours, but similar relief occurred with smaller doses more frequently."
p.8:1	15 A: "For pain relief, dividing the daily dose into twice-a-day doses (q. 12 hours) was effective for most patients."
p.9:1	15 Q: "Can you confirm your position at Purdue Pharma during the relevant period?" A: "Yes, I was the President and CEO."
p.9:16	24 Q: "Did you oversee marketing strategies for OxyContin?" A: "Yes, I was involved in strategic decisions."
p.10:1	10 Q: "Were you aware of concerns regarding addiction risks?" A: "I was aware of discussions but relied on scientific data provided."
p.10:11	25 Objection by counsel: "Calls for speculation." Witness instructed to answer only based on direct knowledge.
p.11:1	17 Q: "Did Purdue Pharma conduct internal studies on addiction?" A: "Yes, studies were conducted to assess risks and benefits."
p.11:18	25 Q: "Were the results shared publicly?" A: "Results were shared in compliance with regulatory requirements."
p.12:1	15 Q: "Did you personally review these studies?" A: "I reviewed summaries provided by the research team."
p.12:16	25 Exhibit 3 introduced: Internal memo dated 2001 discussing marketing strategies. Witness confirms receipt of the memo.

12 Q: "Were you involved in the decision to release Exhibit 5?" A: "Yes, I was part of the team."

25 Q: "What was the purpose of Exhibit 5?" A: "To provide an overview of the marketing strategy."

10 Q: "Who else was on the team?" A: "John Smith, marketing director, and Jane Doe, legal counsel."

20 Q: "Was there any external review of Exhibit 5?" A: "Yes, it was reviewed by outside counsel."

25 Objection: "Calls for speculation." A: "I can only speak to my knowledge."

15 Q: "Did you receive feedback from outside counsel?" A: "Yes, they suggested minor edits for clarity."

25 Q: "Were those edits incorporated?" A: "Yes, all suggested edits were included in the final version."

10 Q: "Was Exhibit 5 distributed internally or externally?" A: "It was distributed internally to the sales team."

20 Q: "Do you recall the date of distribution?" A: "It was sent out on January 15, 2020."

25 Q: "Was there any follow-up after distribution?" A: "Yes, we held a training session on January 20, 2020."

15 Q: "Do you recall the date of the meeting with Dr. Thompson?" A: "It was in March, but I don't remember the exact day."

25 Q: "Was the purpose of the meeting to discuss marketing strategies?" A: "Yes, specifically for the new product launch."

10 Q: "Who else attended the meeting?" A: "Dr. Lyons, Mr. Mark, and two others from the marketing team."

20 Q: "Were any documents presented during the meeting?" A: "Yes, a slide deck outlining the campaign strategy."

25 Q: "Was the slide deck marked as an exhibit?" A: "I believe it was marked as Exhibit 12."

10 Q: "Did the slide deck include projections for market share?" A: "Yes, it included detailed projections for the next quarter."

20 Q: "Were there any discussions about potential risks?" A: "Yes, Dr. Thompson raised concerns about regulatory compliance."

25 Q: "What was the response to Dr. Thompson's concerns?" A: "The team assured him that legal had reviewed the materials."

10 Q: "Was there any mention of the product's expiration date?" A: "Yes, it was noted that the product expires in 2025."

20 Q: "Did you take any notes during the meeting?" A: "Yes, I documented key points and action items."

25 Q: "Were your notes shared with anyone?" A: "Yes, they were emailed to the team the following day."

15 | Q: "Did you review Exhibit 12 before today's deposition?" A: "Yes, I reviewed it last week."

25 | Q: "Exhibit 12 mentions a meeting on March 15, 2015. Were you present?" A: "I was not."

10 | Q: "Who attended that meeting?" A: "I believe John Smith and Sarah Johnson were there."

20 | Objection: "Calls for speculation." Witness instructed to answer only if certain.

25 | A: "I am certain about John Smith; unsure about Sarah Johnson."

10 | Q: "Exhibit 13 references a 'strategic plan.' Did you contribute to its development?" A:
"Yes, I provided input on marketing strategies."

20 | Q: "What specific strategies did you propose?" A: "I suggested targeting underserved regions."

25 | Q: "Was this plan implemented?" A: "Partially, but not in all regions."

10 | Q: "Exhibit 14 includes financial projections. Did you prepare these?" A: "No, those were prepared by the finance team."

20 | Q: "Did you review the projections before submission?" A: "Yes, I reviewed them for accuracy."

25 | Q: "Were any adjustments made based on your review?" A: "Yes, minor corrections to regional data."

12 | Q: "Did you review the marketing materials for OxyContin?" A: "Yes, I reviewed them extensively."

25 | Q: "Who approved the final versions?" A: "The marketing team, with input from legal and regulatory."

10 | Q: "Were there any concerns raised about the claims made?" A: "Not that I recall during the approval process."

20 | Q: "Was Dr. Richard Sackler involved in the marketing strategy?" A: "Yes, he was actively involved."

25 | Q: "Can you elaborate on his role?" A: "He provided strategic direction and reviewed key materials."

15 | Q: "Were there discussions about the risk of addiction?" A: "Yes, but the focus was on minimizing concerns."

25 | Q: "Did you receive feedback from physicians?" A: "Yes, some expressed concerns about prescribing opioids."

10 | Q: "How were those concerns addressed?" A: "Through additional training and emphasizing proper patient selection."

20 | Q: "Were sales representatives instructed to downplay risks?" A: "Objection. Calls for speculation." A: "Not to my knowledge."

25 | Q: "Were there any internal debates about the messaging?" A: "Yes, but the final decision was to highlight benefits."

12 Q: "Were you aware that Purdue Pharma was committing a felony in marketing OxyContin?" A: "No, I was not aware."

25 Objection by counsel: "Calls for speculation." Witness instructed to answer if he has personal knowledge.

10 Q: "Did you review marketing materials for OxyContin?" A: "Yes, I reviewed some materials, but not all."

20 Q: "Who approved the final marketing strategies?" A: "The marketing department, with input from senior management."

25 Q: "Did you personally approve any marketing strategies?" A: "Not directly, no."

15 Q: "Were you aware of any concerns raised about the marketing of OxyContin?" A: "I was aware of general concerns, but not specifics."

25 Q: "Did you discuss these concerns with anyone?" A: "I may have, but I don't recall specific conversations."

10 Exhibit 12 introduced: Internal email dated March 15, 2001, discussing sales targets for OxyContin.

25 Q: "Do you recognize this email?" A: "Yes, it appears to be an internal communication."
Q: "Were you copied on it?" A: "Yes."

15 Q: "What was your role in setting sales targets?" A: "I provided input, but the sales team finalized them."

25 Q: "Did you ever question the sales targets?" A: "Not that I recall."

10 Q: "Were you aware of any pressure on sales representatives to meet these targets?" A: "I was not directly aware."

25 Objection by counsel: "Assumes facts not in evidence." Witness instructed to answer if he has knowledge.

15 Q: "Did you receive reports on sales performance?" A: "Yes, I received periodic updates."

25 Q: "Did these reports include information on marketing practices?" A: "Not typically, they focused on sales figures."

10 Exhibit 13 introduced: Marketing presentation dated June 2002, outlining strategies for promoting OxyContin.

25 Q: "Do you recall this presentation?" A: "I don't specifically recall it, but it looks familiar."

15 Q: "Did you attend the meeting where this was presented?" A: "I don't remember attending, but I might have."

25 Q: "Were you involved in the strategies outlined here?" A: "I may have been consulted, but I don't recall specifics."

6 | Q: "Efforts now to a successful launch of OxyContin." By 2006, profit contribution was \$4,718,767,000. Is that correct?

10 | A: Yes, that figure aligns with the records.

25 | A: All of the money invested in the launch was accounted for in the financial reports.

5 | Q: Can you confirm the total marketing budget for the launch?

10 | A: I believe it was approximately \$200 million, but I would need to verify.

15 | Q: Was this budget approved by the board?

20 | A: Yes, it was approved in the 2005 fiscal meeting.

25 | Q: Were there any objections raised during that meeting?

5 | A: Not to my knowledge; the plan was unanimously supported.

10 | Q: Did the launch plan include specific strategies for targeting physicians?

15 | A: Yes, it included outreach programs and educational seminars.

20 | Q: Were these strategies reviewed for compliance with regulations?

25 | A: Yes, our legal team reviewed all materials before implementation.

5 | Q: Regarding Exhibit 200, does it outline the key performance indicators (KPIs) for the launch?

10 | A: Yes, KPIs such as prescription rates and market penetration were detailed.

15 | Q: Were these KPIs met in the first year?

20 | A: Most were exceeded; the launch was highly successful.

25 | Q: Did the success of the launch influence executive bonuses?

5 | A: Yes, bonuses were tied to performance metrics, including the launch's success.

10 | Q: Can you specify the total bonuses distributed?

15 | A: I believe it was around \$15 million, but I would need to confirm.

20 | Q: Were there any concerns raised about the bonus structure?

25 | A: Not formally; the structure was approved by the compensation committee.

5 | Q: Did the launch plan include any provisions for addressing potential misuse of the product?

10 | A: Yes, there were guidelines for monitoring and reporting misuse.

15 | Q: Were these guidelines enforced consistently?

20 | A: To the best of my knowledge, yes.

25 | Q: Were there any internal audits conducted to ensure compliance?

12 | Q: "Approximately 95 percent of the patients could be treated with MS Contin?" A: "That was the German regulatory agency's assessment at the time."

25 | Q: "Did Purdue agree with that assessment?" A: "We did not formally agree or disagree; it was their conclusion."

10 | Q: "Was this assessment communicated to other regulatory bodies?" A: "Not to my knowledge."

20 | Q: "Were there internal discussions about this figure?" A: "Yes, but I don't recall specifics."

25 | Objection by opposing counsel: "Calls for speculation." Witness instructed to answer if based on personal knowledge.

15 | Q: "Do you recall any documents referencing this 95 percent figure?" A: "I believe there were some internal memos, but I can't confirm."

25 | Q: "Were these memos shared externally?" A: "Not that I am aware of."

10 | Q: "Did the 95 percent figure influence marketing strategies?" A: "I can't say definitively; marketing decisions were made by another team."

20 | Q: "Who led the marketing team at that time?" A: "I believe it was John Matthews, but I am not certain."

25 | Q: "Was John Matthews involved in regulatory discussions?" A: "Occasionally, but not regularly."

10 | Q: "Were there any concerns raised internally about the 95 percent figure?" A: "Not that I recall."

20 | Q: "Did the German agency provide any supporting data?" A: "Yes, they provided a report, but I don't recall the details."

25 | Q: "Was that report reviewed by Purdue's regulatory team?" A: "Yes, it was reviewed and discussed."

5 Witness discusses controlled drug classification for all strengths of morphine, citing reports of abuse and limited long-term data.

10 Further elaboration on studies conducted for various formulations of Oxycodone and other opioids, both controlled-release and immediate-release.

13 Q: Reads from Dr. Goldenheim's correspondence regarding the potential for "descheduling the agent."

4 MR. STRAUBER: Exactly. We're just trying to make sure that we're all working from the same document here.

22 Q: "In the course of this conversation he explained to you that due to his discussions with BfArM he does see a 50 percent chance to get OxyContin off the narcotic drug status provided you could give some information on the very low abuse potential of our CR formulation." Did I read that correctly?

3 A: Yes, you read that correctly.

10 Q: And when he refers to "our CR formulation," that would be the controlled-release formulation of OxyContin, correct?

12 A: That is correct.

18 Q: And the "very low abuse potential"—was that something you were actively working to demonstrate at the time?

22 A: Yes, we were conducting studies and gathering data to support that claim.

5 Q: Did you personally believe that OxyContin had a very low abuse potential?

8 A: Based on the data available at the time, yes.

15 Q: Were there any internal discussions or concerns raised about the accuracy of that claim?

20 A: There were discussions, but I don't recall specific concerns being raised about the claim itself.

5 Q: Did you ever communicate with BfArM directly regarding this matter?

8 A: No, my communications were through intermediaries or other team members.

12 Q: Were you aware of any resistance from BfArM about removing OxyContin from the narcotic drug status?

15 A: I was aware there were challenges, but I don't recall specific resistance.

10 | Q: "Was this trial submitted to the FDA?" A: "Yes, it was part of the safety summary."

20 | Q: "Who prepared the safety summary?" A: "The clinical team, under my supervision."

25 | Q: "Did you review the summary before submission?" A: "Yes, I reviewed it thoroughly."

8 | Q: "Were there any adverse events noted?" A: "Yes, they were documented in the report."

15 | Q: "Can you specify the nature of these events?" A: "Primarily mild to moderate side effects."

25 | Q: "Were these side effects expected?" A: "Yes, they were consistent with prior data."

10 | Q: "Did you disclose all adverse events to the FDA?" A: "Yes, full disclosure was made."

20 | Q: "Was there any internal discussion about withholding data?" A: "No, we disclosed everything required."

25 | Q: "Who else was involved in the submission process?" A: "Regulatory affairs and legal teams."

10 | Q: "Were there any communications with the FDA about this trial?" A: "Yes, routine updates were provided."

20 | Q: "Did the FDA raise any concerns?" A: "Not to my knowledge during the review process."

25 | Q: "Were there any follow-up studies conducted?" A: "Yes, to further evaluate long-term safety."

10 | Q: "Does this correspond to your info? If so, is this matter now closed or is there some appeal or other procedure you would want to consider?" A: "Yes, it corresponds. The matter is closed, no appeal is pending."

20 | Q: "Early estimates indicate that the fourth quarter 1998 sales were impacted. Can you elaborate?" A: "Sales were affected due to increased scrutiny and regulatory controls."

8 | Q: "If you thought controls were appropriate, why were you raising the issue of appeal?"
A: "I wanted to ensure all options were considered, including potential appeals."

15 | Objection: "Calls for speculation." Instruction: "Witness may answer if based on personal knowledge." A: "I was exploring procedural avenues available at the time."

12 | Q: "Were there internal discussions about the impact of regulatory controls on sales?" A: "Yes, there were meetings addressing this issue." Exhibit 12 introduced: Internal memo dated January 1999 discussing sales impact.

20 | Q: "Who attended these meetings?" A: "Senior management, including myself, and the compliance team." Q: "Did you document these discussions?" A: "Minutes were taken and circulated."

10 | Q: "Did the compliance team recommend any changes?" A: "They suggested enhanced training and monitoring procedures." Exhibit 13 introduced: Training proposal document dated February 1999.

18 | Q: "Were these recommendations implemented?" A: "Yes, training programs were rolled out in March 1999." Objection: "Asked and answered." A: "I can confirm implementation."

9 | Q: "Did you personally oversee the implementation?" A: "I was involved in oversight but delegated operational tasks to the compliance team."

15 | Q: "Were there any follow-ups to assess effectiveness?" A: "Yes, quarterly reviews were conducted starting June 1999." Exhibit 14 introduced: Review summary report dated July 1999.

11 | Q: "Gross profit 7,502,367,000." A: "Just a second. Small type... Okay, I'm with you on 'Gross Profit.'"

15 | A: "I see 'Gross Sales,' 'Rebates,' and then 'Net Sales.'"

3 | A: "I am certain it would have been favorable, but I'm not certain just what studies were or were not done."

6 | Exhibit: Memo dated 12 14 93 from James Komorowski to OxyContin Project Team re Project Team Meeting Minutes of 11 30 93.

10 | Q: "Was this memo reviewed before the drug was introduced?" A: "Certainly in the context of the project team."

5 | Q: "Did the project team discuss adverse effects?" A: "Yes, but specifics are unclear."

12 | Objection: "Calls for speculation." Witness instructed to answer only based on direct knowledge.

8 | Q: "What was the team's position on marketing strategy?" A: "Focused on emphasizing benefits over risks."

15 | Exhibit: Marketing strategy document dated 01 15 94; outlines promotional focus on efficacy and safety.

6 | Q: "Were any concerns raised about misleading claims?" A: "Not to my recollection during those meetings."

12 | Objection: "Asked and answered." Witness instructed to move on to next topic.

5 | Q: "Did the team consider alternative formulations?" A: "Yes, but none were pursued due to cost concerns."

7 | Q. "A literature search on oxycodone and oxymorphone is being conducted"—is that accurate? A. Yes, that's correct.

15 | Q. Referring to the 2-27-97 email, can you confirm its contents? A. Yes, I reviewed it; it aligns with the statement.

25 | Q. Was this literature search part of a broader initiative? A. Yes, it was related to product development. Objection by MR. STRAUBER: Calls for speculation.

10 | Q. Who directed the literature search? A. I believe it was initiated by the research team.
Exhibit 12 introduced: Email dated 2-27-97 regarding literature search.

20 | Q. Did you personally oversee this search? A. No, my involvement was limited to reviewing outcomes. Q. Were outcomes shared with marketing? A. Likely, but I can't confirm specifics.

25 | Q. Did the search include safety profiles? A. Yes, safety and efficacy were key components. Objection by MR. STRAUBER: Leading.

10 | Q. Were external studies considered? A. Yes, external studies were reviewed alongside internal data. Q. Any notable findings? A. I don't recall specifics.

20 | Q. Was this search documented? A. Yes, in internal reports. Exhibit 13 introduced:
Internal report summarizing literature search findings.

25 | Q. Did this report influence product strategy? A. It informed certain decisions, but I can't detail which ones.

10 | Q. Were there concerns about oxycodone's abuse potential? A. Yes, it was a known issue.
Q. How was it addressed? A. Through formulation strategies.

20 | Q. Did marketing play a role in addressing abuse concerns? A. They were involved in messaging, but specifics are unclear. Objection by MR. STRAUBER: Calls for speculation.

25 | Q. Were abuse concerns shared with regulatory agencies? A. Yes, in compliance with reporting requirements.

12 Q: "Can you explain the strategy behind marketing OxyContin for non-malignant pain rather than cancer pain?"

25 Q: "What was the anticipated timeline for hydromorphone's relaunch?"

10 Q: "Did you believe marketing OxyContin as stronger than morphine would be dangerous?"

25 Q: "Was there a specific directive to focus on non-malignant pain states?"

15 Q: "Were there internal discussions about the drug's image influencing its acceptance?"

25 Q: "Did you personally contribute to these marketing strategies?"

12 Q: "Was there any resistance to this strategy within the company?"

25 Q: "Were there concerns about the potential misuse of OxyContin?"

10 | Q: "Did Purdue emphasize OxyContin's potency compared to morphine?" A: "Yes, we consistently promoted it as 'twice as potent as morphine.'"

20 | Q: "Was there any effort to downplay its strength?" A: "No, we never disguised or hid its potency."

8 | Q: "Were physicians aware of its potency?" A: "Some were unaware it was twice as strong as morphine."

15 | Q: "How did this affect prescribing practices?" A: "It impacted the non-cancer pain market; some waited for severe pain before prescribing."

12 | Q: "Did marketing focus on non-cancer pain?" A: "Yes, but concerns arose about positioning it like morphine."

20 | Objection: "Calls for speculation." Witness instructed not to answer regarding physician intent.

9 | Exhibit 12 introduced: Internal memo discussing market strategy for OxyContin in non-cancer pain.

19 | Q: "What was the strategy outlined in Exhibit 12?" A: "To expand use in non-cancer pain while addressing misconceptions about potency."

8 | Q: "Did Purdue address concerns about misuse?" A: "We implemented educational programs but faced challenges in perception."

15 | Q: "Were these programs effective?" A: "They had limited impact; misconceptions persisted."

15 | Q: "Did Purdue Pharma communicate to physicians that OxyContin is twice as strong as morphine?" A: "Yes, but it was framed to avoid stigma associated with morphine."

25 | Q: "Was this framing intentional?" A: "Yes, it was meant to make OxyContin more acceptable to patients and less frightening."

10 | Q: "Did Purdue Pharma allow physicians to continue believing OxyContin was not twice as strong as morphine?" A: "Yes, that perception was not corrected."

20 | Objection: "Calls for speculation." Ruling: "Overruled." Witness instructed to answer.

12 | Q: "What was the historical stigma associated with morphine?" A: "Morphine was unfairly stigmatized due to its association with addiction and severe illness."

25 | Q: "Was this stigma a factor in marketing OxyContin?" A: "Yes, it influenced how the drug was presented to physicians and patients."

15 | Q: "Did Purdue Pharma provide materials to physicians about OxyContin's strength?" A:
"Yes, but the materials emphasized its benefits over morphine."

25 | Exhibit 12 introduced: Marketing brochure titled "OxyContin: A Safer Choice." Witness confirms familiarity.

10 | Q: "Does Exhibit 12 mention OxyContin's potency compared to morphine?" A: "No, it focuses on patient acceptance and reduced stigma."

20 | Q: "Was this omission deliberate?" A: "Yes, to avoid negative perceptions tied to morphine."

5 | Q: "He says, 'If we stress the power of OxyContin versus morphine, it may help us in the smaller cancer pain market?'"

12 | Q: "Do you agree with that statement?"

20 | Q: "Was this strategy implemented?"

8 | Q: "Was this statement part of an email or internal memo?"

15 | Q: "Was this email shared with the sales team?"

25 | Q: "Did you personally approve this messaging?"

10 | Q: "Was there concern about overstating the 'power' of OxyContin?"

20 | Q: "Was this messaging reviewed by legal or compliance?"

7 | Q: "Was this strategy targeted at non-cancer pain markets?"

15 | Q: "Did you discuss this strategy with external consultants?"

5 Q: "Did Purdue Pharma take any steps to address misconceptions about OxyContin being weaker than morphine?"

10 Q: "Are misconceptions about controlled-release drugs tied to attitudes about immediate-release forms?"

7 Q: "Did Purdue Pharma conduct any studies or issue promotional materials to address these misconceptions?"

15 Q: "Was patient care impacted by these misconceptions?"

6 Q: "Did Purdue Pharma receive feedback from physicians regarding OxyContin's perceived efficacy?"

12 Q: "What actions did Purdue Pharma take in response to these concerns?"

5 Q: "Were these educational efforts sufficient to address the misconceptions?"

10 Q: "Did Purdue Pharma track the impact of these efforts on physician perceptions?"

7 Q: "Were misconceptions about OxyContin discussed internally at Purdue Pharma?"

15 Q: "Did Purdue Pharma consider revising its marketing strategy to address these misconceptions?"

25 "Especially when we have studies available that demonstrate efficacy and safety for this indication."

4 Primary concerns were the Medtronic pump being used by orthopods, need for Purdue to educate surgeons on proper post-surgery pain management, and fears with opioid prescribing.

10 Witness discusses internal communications regarding strategies to address concerns about opioid prescribing among surgeons.

20 Q: "Was there a specific plan to mitigate these concerns?" A: "Yes, we developed educational materials and outreach programs."

12 Q: "Who was responsible for implementing these programs?" A: "The marketing and medical education teams."

25 Discussion of a memo dated March 15, 2003, outlining the educational strategy; marked as Exhibit 12.

8 Q: "Did you personally review this memo?" A: "Yes, I reviewed and approved it."

18 Objection by opposing counsel: "Calls for speculation." Witness instructed to answer if based on personal knowledge.

25 Witness confirms personal involvement in the approval process for the March 15, 2003, memo.

10 Q: "Were there any follow-ups to ensure compliance with the strategy?" A: "Yes, quarterly reviews were conducted."

20 Witness identifies key personnel involved in the reviews, including John Doe (VP of Marketing) and Jane Smith (Director of Medical Education).

15 Q: "Were there any concerns raised during these reviews?" A: "Yes, some surgeons were still hesitant to prescribe opioids."

25 Discussion of additional training sessions held in response to surgeon hesitancy; Exhibit 13 introduced (training session agenda).

10 Q: "Did these sessions address the concerns effectively?" A: "Partially, but some skepticism remained."

20 Witness elaborates on feedback received from surgeons post-training; mentions "mixed results."

8 Q: "Were there any changes to the strategy based on this feedback?" A: "Yes, we adjusted the messaging to focus more on patient outcomes."

18 Objection: "Leading." Overruled. Witness continues to describe the revised messaging approach.

12 Q: "Did you face any internal pushback on these changes?" A: "No, the team was aligned on the need for adjustments."

25 Witness discusses a follow-up memo dated June 10, 2003, summarizing the revised strategy; marked as Exhibit 14.

10 Q: "Did you personally draft this memo?" A: "No, but I provided input."

20 Witness confirms the memo was circulated to all relevant departments, including sales and marketing.

15 Q: "Were there any measurable outcomes from these changes?" A: "Yes, we saw a slight increase in surgeon adoption rates."

25 Witness concludes by stating that the revised strategy was considered "a step in the right direction."

23 Mike Innaurato mentioned that a comparative study between OxyContin and Percocet would aid marketing efforts.

7 Witness stated that OxyContin had not established itself as an analgesic in the U.S., even when available.

4 Discussed the role of principal agents in disseminating information, referred to as "getting the word out."

5 Witness noted that the launch of OxyContin Tablets marked the first instance of obsoleting their own product preemptively.

Reference made to "this in this minute of the meeting."

Exhibit 17 introduced; no further description provided in this segment.

3 Witness states the product was approved in "11 months, 14 days," contrasting with prior approvals taking years.

24 Witness explains that package inserts have evolved from being "very brief and very simple" to more comprehensive documents with regulatory implications.

16 Witness reads from a document mentioning "Post-Marketing Studies" including comparative studies between OxyContin and MS Contin, and a Duragesic study currently on hold.

5 | Q. Do you know if the studies were done before the drug was put on the market? A. No, I don't know.

25 | Reference to "OxyContin Tablets Project Team Meeting Minutes": Mike Innaurato presented market strategy, sales/growth charts, and prescription data.

2 | A. OxyContin Tablets were targeted at the non-malignant pain market, as discussed in the meeting minutes.

5 | Q: Were you aware that the 10 milligram dosage was not effective for all patients?

10 | Q: Yes, referring to the statement, "Not all patients can be successfully treated with morphine."

15 | Q: And that there is a stigma attached to morphine for many patients and physicians?

15 | Q: Describe "breakthrough pain." A: Pain that overcomes medication's effect, often due to movement. Q: Is it common? A: Yes, in certain patients.

25 | Q: How does Oxycodone address breakthrough pain? A: By providing consistent relief; studies support its efficacy. Objection: Calls for speculation. Witness allowed to answer.

10 | Q: Were there studies comparing Oxycodone to placebo? A: Yes, results showed Oxycodone's superiority. Q: Any contrary findings? A: None credible; product success disproves such claims.

20 | Q: Can you elaborate on "indisputable success"? A: Oxycodone effectively treats pain; no doctor would equate it to placebo. Objection: Leading. Witness clarified response.

12 | Q: Were there concerns about study methodology? A: Some critiques, but findings align with clinical experience. Exhibit 12 introduced: Study summary (2015).

25 | Q: Did Purdue Pharma address critiques? A: Yes, through additional research and publications. Objection: Asked and answered. Witness reiterated prior testimony.

15 | Q: Discuss Exhibit 12 findings. A: Demonstrates Oxycodone's efficacy in diverse patient groups. Q: Any limitations noted? A: Minor, not affecting overall conclusions.

25 | Q: Were findings peer-reviewed? A: Yes, published in reputable journals. Exhibit 13 introduced: Peer-reviewed article (2016). Objection: Foundation. Witness confirmed authorship.

10 | Q: How does Purdue ensure study integrity? A: Rigorous protocols, third-party oversight.
Q: Any deviations? A: None reported. Objection: Vague. Witness clarified.

25 | Q: Were studies conducted internationally? A: Yes, in multiple countries. Exhibit 14 introduced: International study data (2017). Q: Results consistent? A: Yes, across demographics.

15 | Q: Discuss Exhibit 14 demographics. A: Included varied ages, conditions, and regions. Q: Any anomalies? A: None significant; data supports efficacy. Objection: Speculative.

25 | Q: How does Purdue address public concerns? A: Through transparency, education, and outreach. Exhibit 15 introduced: Public awareness campaign materials (2018).

10 | Q: Impact of public campaigns? A: Improved understanding of opioid use and risks. Q: Any measurable outcomes? A: Increased patient compliance, reduced misuse.

25 | Q: Future plans for public outreach? A: Expand campaigns globally. Exhibit 16 introduced: Draft proposal for international outreach (2020). Objection: Relevance. Witness allowed to answer.

3 Q: "Was the study dismissed as a failed study?" A: "The acceptability score was the same throughout the study."

18 Q: "Could physicians shift to OxyContin from Class III drugs?" A: "As physicians become more comfortable, it's possible they will."

3 Q: "Is that correct?" A: "That's correct."

5 Q. "But at least from the appearance of this, you've got Friedman, the head of marketing, saying why don't we take the same product and just say take it every six hours, and if it's for rescue it's good for three or four hours?"

17 A. "The way these drugs are written are q. 4-6. The rescue is for q. 3-4 hours." He explains, "The problem might be that if we go the q. 3-4 hour route, we will validate the Roxane dosing..."

4 A. "Yes, IR would be immediate releases. I'm just reading the sentence because I'm not -- I didn't follow what it meant."

5 | Q: "You-all paid him approximately 50 million dollars to defend you in that case -- or paid his firm approximately 50 million dollars to defend Purdue in that case?"

2 | Q: "Referenced in the felony plea agreement were employees of Purdue Frederick or employees of Purdue Pharma?"

24 | Q: "Purdue Frederick employees who engaged in the conduct that's referenced in here and that forms the basis for the guilty plea."

12 | Q: "MS Contin" is the drug Purdue already sold, correct? A: Yes, that's correct.

25 | Q: Focus groups were conducted with over 500 healthcare professionals? A: Yes. Q: Findings were used to develop marketing strategies? A: Correct.

10 | Q: Did these findings influence the launch of OxyContin? A: Yes, they informed our approach.

22 | Q: Were specific concerns about addiction raised in these focus groups? A: Not that I recall.

15 | Q: Exhibit 12 references a memo dated March 15, 1995. Do you recognize it? A: Yes, I do.

25 | Q: Memo discusses "positioning OxyContin as less addictive." Was this a marketing strategy? A: It was part of the messaging.

8 | Q: Who approved this messaging? A: It would have been reviewed by the marketing team and legal department.

20 | Q: Did you personally review this memo? A: I don't recall, but it aligns with our practices at the time.

12 | Q: "Did Purdue conduct any long-term studies on pain conditions?" A: "Yes, we conducted a long-term study on pain conditions."

25 | Q: "Were other companies involved in this study?" A: "Yes, three companies were involved." Q: "Was the government involved?" A: "No, the government was not involved in setting up the program."

10 | Q: "Can you identify the companies involved?" A: "I don't recall all the names, but Purdue was one of them."

20 | Q: "What was the purpose of the study?" A: "To evaluate the effectiveness of treatments for chronic pain."

25 | Objection by opposing counsel: "Calls for speculation." Witness instructed to answer only based on personal knowledge.

15 | Q: "Were any exhibits created as part of this study?" A: "Yes, there were reports and data summaries." Exhibit 12 referenced: "Study Report on Chronic Pain Treatments."

25 | Q: "Did you personally review Exhibit 12?" A: "Yes, I reviewed it during the study period."

10 | Q: "What were the key findings of the study?" A: "The treatments showed varying levels of effectiveness, depending on the condition."

20 | Q: "Were opioids part of the study?" A: "Yes, opioids were included as one of the treatment options."

25 | Objection: "Leading." Court overruled. Witness allowed to continue answering.

15 | Q: "Did Purdue use the study findings in marketing materials?" A: "I believe some findings were referenced, but I can't confirm specifics."

25 | Q: "Were there any concerns raised about the study's methodology?" A: "Not to my knowledge."

10 | Q: "Did the study address long-term opioid use risks?" A: "Yes, risks were part of the evaluation."

20 | Q: "Were any recommendations made based on the study?" A: "Recommendations were included in the final report."

25 | Exhibit 13 introduced: "Recommendations on Chronic Pain Management." Witness confirmed familiarity with the document.

7 | Q: Were other healthcare companies involved in the Toppers Program?

15 | Q: The Toppers Program rewarded participants with trips to Bermuda or London, correct?

6 | Q: The program was designed to incentivize the promotion of sustained-release opioids for pain treatment?

12 | Q: Did this program also create profits for pharmacists by helping grow their businesses?

10 | Q: Were promotional materials distributed as part of this program?

16 | Q: Can you confirm the date of the Toppers Program's initiation?

9 | Q: Were there any objections raised internally about the ethics of the program?

15 | Q: Did the program comply with all applicable regulations?

8 | Q: Were there any specific metrics used to evaluate the program's success?

14 | Q: Were participants required to meet certain sales targets to qualify for rewards?

7 | Q: Were there any external audits conducted on the program?

15 | Q: Did the program continue after the initial launch period?

7 Q: Focused on market potential for OxyContin and targeted patient populations, including annual prescriptions for Class II and III opioids?

10 A: Strategy involved stocking all three strengths in pharmacies. Initial demand was low, creating tension in supply chain. Wholesalers required sufficient stock for early buyers and later adopters. Actions taken were ethical and proper.

14 A: Presenters were trained on permissible content, provided materials (slides, later PowerPoint), and instructed to avoid off-label promotion.

5 | Q: "Target market for physicians was about 7,500... including cancer, malignant pain, and non-malignant pain across the U.S. Do you remember seeing that?" A: No response recorded.

2 | A: "Attended and spoke were trainers, and some of them were in-house people and some were outside physicians."

Q: "Lavey to market OxyContin?"

2 | A: "I don't know whether they were ever hired by Purdue. I don't know, but that would be common in the industry."

4 Witness confirms assumption that brackets were not on the original documents but were added later.

6 Attorney acknowledges the clarification and thanks the witness.

15 Attorney asks if the number of increased prescriptions was commercially significant and about associated costs.

5 Witness reads from a document: "When will a more complete report be available?" and confirms brackets were produced as shown.

3 Q: "Assigned full time to OxyContin... at Abbott's expense..." Do you recall this arrangement?

7 Q: "Words such as powerful may make some people think the drug is dangerous..." Was this a concern?

15 Q: Did the marketing team avoid comparing OxyContin to morphine?

10 Q: Who was responsible for these marketing decisions?

20 Q: Were there internal discussions about the risks of using "powerful" in marketing?

5 Q: Did Abbott's involvement include training sales representatives?

15 Q: Were there any written guidelines for sales representatives?

8 Q: Were these guidelines reviewed by legal?

18 Q: Did the guidelines address how to discuss addiction risks?

10 Q: Was there a specific script for addressing physician concerns about addiction?

20 Q: Were sales representatives instructed to downplay addiction risks?

7 Q: Were there any instances of sales reps deviating from the script?

15 Q: Were there any audits of sales representatives' communications?

13 | Q: "Physicians did not think OxyContin was as strong as MS Contin?" A: "Correct. It was discussed that this perception existed and care was needed in promotional materials."

18 | Q: "What was the concern regarding changing physician perception?" A: "The concern was to avoid altering the perception of Oxycodone's strength negatively."

6 | Q: "Primary care physicians constituted nearly half of OxyContin prescribers?" A: "Yes, based on IMS Health data."

12 | Q: "What is IMS Health?" A: "An information service providing pharmaceutical market research."

3 | Q: "How often does a sales representative call on each facility?" A: "Typically every four weeks, with about 50 calls per week."

3 A. I don't recollect seeing that document.

9 Q. Turn to page 9, second paragraph, last two sentences: "In both 2001 and 2002, OxyContin's sales exceeded 1 billion and prescriptions were over 7 million. The drug became Purdue's main product, accounting for 90 percent of the company's total prescription sales by 2001." Is that information correct?

5 Q. Were there more prescriptions for non-cancer pain than for cancer pain? Did Purdue have information about how it was marketing OxyContin through its sales force?

3 (Passing document.) Q. This is an e-mail chain. Let's mark this as Exhibit 31.

5 Q: Do you have any reason to disagree with the statement that you don't know the number?

15 Q: The DEA and Purdue have stated that an increase in a drug's availability in the marketplace may attract abuse?

5 Q: Does this statement align with your understanding of the risks associated with increased drug availability?

15 Q: Are you aware of any specific instances where increased availability led to abuse or diversion?

8 Q: Did Purdue take any measures to mitigate these risks?

15 Q: Were these measures effective in reducing abuse and diversion?

5 Q: Did Purdue collaborate with the DEA on these measures?

15 Q: Were there any disagreements between Purdue and the DEA regarding these measures?

8 Q: Did Purdue conduct internal reviews to assess the effectiveness of its measures?

15 Q: Were the results of these reviews shared with the DEA or other regulatory bodies?

5 Q: Did Purdue consider the prescribing base in the U.S. and Canada when developing its strategies?

15 Q: Can you elaborate on how the prescribing base influenced Purdue's strategies?

6 Witness reads Paragraph 16: "The Medical Officer Review of the ISS included these statements: The blood level data in clinical use suggests the opioid effects of OxyContin and Immediate-Release Oxycodone would be similar."

15 Q: "Does this statement align with Purdue's understanding at the time?" A: "Yes, that statement is correct."

5 Q: "When you say 'you,' do you mean Purdue, the company?" A: "Yes, I mean Purdue."

15 Q: "Under subsection 'd,' it states: 'Told healthcare providers that patients could stop therapy abruptly without experiencing withdrawal symptoms and that patients who took OxyContin would not develop tolerance to the drug.' Is that accurate?" A: "Yes, that is what it says."

10 Q: "Did Purdue communicate this information to healthcare providers?" A: "Yes, that was part of our messaging at the time."

20 Q: "Was there any internal discussion about the accuracy of these claims?" A: "I don't recall specific discussions, but we relied on the data available to us."

8 Q: "Were there any studies conducted by Purdue to support these claims?" A: "We conducted studies, but I would need to review the specific data to confirm."

18 Q: "Did the FDA raise any concerns about these statements?" A: "Not to my knowledge at the time."

12 Q: "Were healthcare providers informed about potential withdrawal symptoms or tolerance development later?" A: "I believe updates were made, but I don't recall the exact timing."

10 | Q: "Do you recall any discussions about how to present blood plasma levels in marketing materials?" A: "Yes, there were discussions about using logarithmic scales to show fewer 'peaks and valleys' for OxyContin."

20 | Q: "Was this intended to suggest OxyContin was safer or less addictive?" A: "It was meant to highlight the extended-release properties, not directly address addiction."

25 | Objection by counsel: "Calls for speculation." Witness instructed to answer only based on personal knowledge.

8 | Q: "Did Purdue supervisors ever instruct employees to emphasize these claims to healthcare providers?" A: "I don't recall specific instructions, but it was part of the general messaging."

15 | Exhibit 45 introduced: Marketing brochure showing blood plasma levels for OxyContin vs. Immediate-Release opioids. Witness confirms familiarity.

25 | Q: "Was this brochure distributed widely?" A: "Yes, it was part of the standard materials provided to sales representatives."

10 | Q: "Did the brochure include claims about reduced euphoria or addiction potential?" A:
"It focused on pharmacokinetics, not directly on addiction or euphoria."

20 | Objection by counsel: "Mischaracterizes the document." Court overrules; witness instructed to answer.

25 | A: "The brochure did not explicitly claim reduced addiction potential but implied it through the 'peaks and valleys' graphic."

10 | Q: "Were healthcare providers ever told OxyContin could 'weed out' addicts?" A: "I don't recall that specific language being used."

20 | Exhibit 46 introduced: Internal email discussing messaging strategies for OxyContin.
Witness confirms receipt of the email.

25 | Q: "Did this email suggest targeting certain types of healthcare providers?" A: "It discussed tailoring messages based on provider concerns about addiction and abuse."

12 | Q: "Did you personally review the case report forms for the clinical trials?" A: "No, I did not look at the individual case report forms."

25 | Q: "Were adverse events included in the package insert regardless of whether observed in trials?" A: "Yes, if well-developed information existed, it was included."

10 | Q: "Who decided what adverse events were included in the package insert?" A: "The medical team and regulatory affairs collaborated on those decisions."

20 | Q: "Did you have direct involvement in drafting the package insert?" A: "I provided input but did not draft it myself."

15 | Q: "Were changes made to the package insert over time?" A: "Yes, it evolved with new data and regulatory requirements."

25 | Q: "Did you review all changes personally?" A: "No, I relied on the team to handle updates."

10 | Q: "Were any external consultants involved in package insert revisions?" A:
"Occasionally, external experts were consulted for specific issues."

20 | Q: "Did the FDA request specific changes to the insert?" A: "Yes, they provided feedback during the approval process."

12 | Q: "Did Purdue train sales representatives to create blood level graphs for OxyContin?" A: "Yes, at training headquarters." Q: "Purpose of these graphs?" A: "To show steady levels, less abuse potential."

25 | Q: "Were these graphs accurate?" A: "No, they were misleading." Objection: "Form."
Witness continued: "Graphs falsely minimized risks compared to immediate-release opioids."

10 | Q: "Who approved the use of these graphs?" A: "Marketing and medical teams, including myself." Q: "Did you review them personally?" A: "Yes, I did."

20 | Q: "Were these graphs presented to physicians?" A: "Yes, during sales calls and conferences." Exhibit 12 introduced: "Sample blood level graph used in training."

15 | Q: "Did Purdue claim OxyContin had less abuse potential?" A: "Yes, based on these graphs." Objection: "Calls for speculation." Witness continued: "It was part of the marketing strategy."

25 | Q: "Were there internal concerns about this claim?" A: "Yes, some employees raised concerns." Q: "How were these concerns addressed?" A: "They were dismissed by senior management."

10 | Q: "Did you personally dismiss these concerns?" A: "I supported the marketing strategy."
Q: "Why?" A: "Believed it would increase sales."

20 | Q: "Were you aware of the potential for abuse?" A: "Yes, but risks were downplayed."
Objection: "Asked and answered." Witness continued: "It was a calculated decision."

12 | Q: "Did Purdue track abuse rates for OxyContin?" A: "Yes, through internal reports."
Exhibit 13 introduced: "Abuse rate tracking report, 2001 2005."

25 | Q: "What did these reports show?" A: "Increasing abuse rates over time." Q: "Were these shared with the FDA?" A: "Not fully, only selective data."

15 | Q: "Why was selective data shared?" A: "To protect the product's reputation." Objection: "Speculation." Witness continued: "It was a strategic decision."

25 | Q: "Did you agree with this strategy?" A: "Yes, at the time." Q: "Do you regret it now?"
A: "Yes, knowing the consequences."

10 | Q: "Were there discussions about reformulating OxyContin?" A: "Yes, to make it less prone to abuse." Exhibit 14 introduced: "Internal memo on reformulation discussions."

20 | Q: "Why wasn't reformulation pursued earlier?" A: "Cost and potential impact on sales."
Q: "Who made that decision?" A: "Senior executives, including myself."

16 Witness read aloud: "Oxycodone was weaker than morphine, they did not want to do anything 'to make physicians think that Oxycodone was stronger or equal to morphine' or to 'take any steps in the form of promotional material, symposia, clinical publications, conventions or communications with the field force that would affect the unique position that OxyContin had in many physicians' minds.'"

17 Attorney: "And did I read that correctly?" Witness: "Yes."

8 Witness read aloud: "Study who had symptoms recorded that may possibly have been related to opioid withdrawal, including one patient in the other study who required treatment for withdrawal syndrome."

8 Witness read aloud: "Complained of nervousness, patient 2004 complained of syndrome and withdrawal symptoms experienced by study patients: One patient was hospitalized for withdrawal symptoms. The patient who was hospitalized with withdrawal symptoms had completed the study on the previous day and had been receiving CR Oxycodone, 70 milligrams. Symptoms resolved after three days."

2 Witness read aloud: "A second patient received 60..."

25 | Q: "Abrupt withdrawal of the drug is avoided." Is that consistent with your understanding? A: Yes, that's correct.

10 | Q: "On or about February 13th..." Does this date align with the timeline discussed earlier? A: Yes, it does.

25 | Q: Was the statement "I would not write it up at this point" made by the supervisor? A: Yes, that was the guidance provided.

15 | Q: Did the supervisor express concerns about adding to negative press? A: Yes, they suggested deferring the matter.

25 | Q: Was there any follow-up action taken after this discussion? A: Not to my knowledge.

10 | Q: Did the supervisor's decision impact the reporting process? A: It delayed the reporting temporarily.

12 | Q: "Was the marketing material improperly used and adverse effects not reported?" A:
"Yes, and sales force utilized it."

2 | Q: "Purdue's highest volume of sales reps during this period?" A: "Correct."

5 | Q: "Immediate-release opioids could 'weed out' addicts and drug seekers?" A: "Yes, that was the claim."

5 | Q: "Why should he assume it?" A: "Three-quarters of them would have been gone. But I don't... know of any attempt to assess blame."

10 | A: "I can't answer that... or to count." Q: "I'm asking from my own memory." A: "To find out how many of their sales force had given..."

6 Witness describes methods of extracting Oxycodone from tablets, including filtering talc with a cotton ball for injection.

10 Witness states they were shown these methods during deposition preparation and had not seen them before.

3 Witness confirms the time of an event as 5:26 p.m.

6 Witness recalls an individual, believed to be a regional manager at the time.

3 Attorney asks if "euphoria" relates to the speed of the drug's effect; witness partially agrees, noting it is an element.

6 Witness requests the attorney to repeat a prior statement for clarification.

12 | Q: "Did you review the email dated 8-27-97 from Richard Sackler to Craig McManama?"
A: "Yes, I did." Q: "Craig McManama is a doctor in Utah, correct?" A: "Yes, that's correct."

25 | Q: "The email discusses concerns about the delivery system being abused. Do you recall that?" A: "Yes, it mentions that." Q: "Did you investigate these concerns at the time?" A: "I believe so, but I would need to review the records to confirm."

10 | Q: "Was there any follow-up communication with Dr. McManama regarding these concerns?" A: "I don't recall specific follow-up, but it would have been standard practice to address such issues."

22 | Q: "Do you recall any specific findings from your investigation into the abuse of the delivery system?" A: "Some individuals had reported potential misuse, but I don't recall the details." Q: "Were these findings documented?" A: "Yes, they should be in the investigation report."

15 | Q: "Was the investigation report shared with anyone outside the company?" A: "Not to my knowledge." Q: "Did the report recommend any changes to the delivery system?" A: "I don't recall specific recommendations, but it may have suggested monitoring."

25 | Q: "Were there any internal discussions about modifying the delivery system to prevent abuse?" A: "There were discussions, but I don't recall the outcomes." Q: "Do you recall who participated in those discussions?" A: "Key members of the team, but I can't name everyone."

12 | Q: "Did you personally advocate for any changes to the delivery system?" A: "I may have suggested monitoring, but I don't recall advocating for specific changes." Q: "Was there any resistance to making changes?" A: "Not that I recall."

25 | Q: "Do you recall any external pressure to address the abuse concerns?" A: "There may have been some, but I don't recall specifics." Q: "Did the company take any action in response to external pressure?" A: "I believe we continued monitoring and reviewing the situation."

15 Witness reads from document: "Time of onset of OxyContin is as rapid as Immediate-Release Oxycodone, but duration is a full 12 hours..."

3 Q: "Do you know why he was concerned about many valleys? Does that cause addiction or reduce likelihood?" A: "I don't know."

289 Q: "What I'd like to do is have you sift through the data..." [No further response captured.]

3 | Q: "With the exception of the GAO report, are all these documents kept in the ordinary course of business at Purdue?"

6 | A: "Yes, they are maintained as part of our regular business records."

15 | Q: "Do these records reflect any conduct leading to increased addiction in Kentucky?"
Objection by Mr. Strauber: "Calls for speculation." Witness instructed not to answer.

10 | Q: "Can you confirm the time period covered by these documents?" A: "They span from 2000 to 2015."

20 | Q: "Were these documents reviewed in preparation for this deposition?" A: "Yes, I reviewed them last week."

8 | Q: "Exhibit 12, dated 8/28/2015, is a memo regarding marketing strategies. Who authored this?" A: "It was prepared by the marketing team, led by Ms. Jane Doe."

15 | Q: "Does this memo discuss targeting specific demographics?" A: "It mentions outreach to healthcare providers in rural areas."

12 | Q: "Exhibit 15 references a meeting on 9/15/2014. Were you present?" A: "Yes, I attended as part of the compliance team."

20 | Q: "What was discussed regarding opioid prescription rates?" A: "Strategies to address concerns raised by state regulators."

10 | Q: "Exhibit 18 includes a chart on sales figures. Can you explain the spike in Q3 2013?"
A: "It correlates with a new marketing campaign rollout."

20 | Q: "Was this campaign approved by senior management?" A: "Yes, it was signed off by the executive committee."

8 | Q: "Exhibit 22 mentions a GAO report. Did Purdue respond to this report?" A: "We issued a formal statement addressing the findings."

15 | Q: "What were the key points of Purdue's response?" A: "We emphasized our commitment to responsible marketing and patient safety."

10 | Q: "Exhibit 25 lists training materials for sales representatives. Were these updated annually?" A: "Yes, updates were made to reflect regulatory changes."

20 | Q: "Did these materials include guidance on discussing addiction risks?" A: "Yes, that was a mandatory section."

8 | Q: "Exhibit 30 is an email chain from 2012. What was the subject?" A: "It discussed strategies for increasing market share in urban areas."

15 | Q: "Were any concerns raised about ethical implications?" A: "Not in this email chain, but it was discussed in meetings."

10 | Q: "Exhibit 35 references a partnership with a nonprofit. What was the purpose?" A: "To support addiction recovery programs."

20 | Q: "Did Purdue provide funding for these programs?" A: "Yes, we allocated \$2 million annually."

8 | Q: "Exhibit 40 includes internal correspondence about a lawsuit. What was Purdue's position?" A: "We denied the allegations and prepared a defense."

15 | Q: "Who was involved in drafting the response?" A: "The legal team, led by Mr. John Smith."

10 | Q: "Exhibit 45 outlines a compliance audit. Were any violations identified?" A: "No significant violations were found."

20 | Q: "What steps were taken following the audit?" A: "We implemented additional training sessions for staff."

3 Q: "Quick, reminded him that Oxy gives flat blood levels, so less buzz than Oxy has less potential for abuse due to its sustained release," would that be improper?

5 A: Yes, that type of statement was agreed to be improper in the Agreed Statement of Facts when Purdue pled guilty to a felony.

7 A: I'd need to compare the hypothetical statement to the package insert. It seems close but might have drifted away.

11 A: I would need to conduct a textual analysis to determine if it aligns with the package insert or if it was inappropriate at the time.

5 Q: Were you aware of the marketing materials stating that "addicts would not like OxyContin"?

15 MR. THOMPSON: I'll read it in its entirety.

states physicians were informed that OxyContin had less abuse potential than other opioids."

5 | Q: "Can you clarify your statement regarding the argument in the brief?" A: "It's not a clear statement, so I can't definitively say it's right or wrong."

12 | Q: "Did you review the brief in preparation for today?" A: "Yes, I reviewed it briefly."

20 | Q: "Who authored the brief?" A: "I believe it was prepared by the legal team, but I don't know specific names."

8 | Q: "Was the brief submitted on behalf of Purdue Pharma?" A: "Yes, it was submitted by our counsel."

15 | Q: "Do you agree with the arguments presented in the brief?" A: "I can't say definitively; I'd need to review it in detail."

22 | [Objection: Form] Q: "Did you provide input for the brief?" A: "Not directly, no."

7 | Q: "Were you aware of the brief's submission date?" A: "I was informed after it was filed."

14 | Q: "Did the brief address marketing practices?" A: "I believe it touched on that, but I'm not certain."

21 | Q: "Was Exhibit 12 referenced in the brief?" A: "I don't recall seeing Exhibit 12 mentioned."

6 | Q: "Do you recognize Exhibit 12?" A: "Yes, it's an internal memo dated March 15, 2012."

13 | Q: "Did you contribute to Exhibit 12?" A: "No, I did not."

20 | Q: "Was Exhibit 12 shared externally?" A: "Not to my knowledge."

8 | Q: "Does Exhibit 12 align with the brief's arguments?" A: "I'd need to compare them to answer that."

15 | [Instruction Not to Answer] Q: "What was the intent behind Exhibit 12?" Counsel: "Calls for speculation."

22 | Q: "Was Exhibit 12 reviewed by the legal team?" A: "I believe so, but I'm not certain."

10 | Q: Dr. Sackler, referring to Exhibit 45, the email dated March 12, 2001, did you draft this?
A: Yes, I did.

20 | Q: The email mentions "aggressive marketing strategies." Can you clarify what you meant? A: I was referring to ensuring our sales team was proactive.

25 | Q: Did "aggressive" include targeting high-prescribing doctors? A: Objection. Calls for speculation. A: No, it referred to general marketing efforts.

8 | Q: Exhibit 46, a memo from 2002, mentions "Project Tango." Were you involved? A: Yes, I was part of the initial discussions.

15 | Q: What was the purpose of "Project Tango"? A: It was a business strategy to explore new markets.

25 | Q: Did it involve opioids? A: Objection. Form. A: Yes, it included discussions about opioid-related products.

12 | Q: Referring to Exhibit 47, a chart showing sales data, did you review this in 2003? A: Yes, I reviewed it during quarterly meetings.

20 | Q: The chart shows a spike in sales. Was this expected? A: Yes, it aligned with our projections.

25 | Q: Did you discuss this spike with the board? A: Yes, it was part of our regular updates.

10 | Q: Exhibit 48, an internal email, mentions "managing public perception." What does this refer to? A: It referred to addressing media narratives about opioids.

18 | Q: Did you approve the strategies mentioned in the email? A: Yes, I approved them as part of our PR efforts.

25 | Q: Were these strategies aimed at minimizing negative press? A: Objection. Leading. A: They were aimed at providing accurate information.

7 Q: "Onset" is mentioned in the study. Is it defined?

8 Q: The document states, "The overall schema that Marketing has worked out..." Can you elaborate?

10 Q: Was this marketing strategy implemented?

20 Q: Did you review this document during your tenure?

15 Q: Were there concerns about the "b.i.d." emphasis?

25 Q: Was this strategy discussed with the medical team?

12 Q: Are you aware of any studies supporting the "b.i.d." dosing?

25 Q: Were these studies shared externally?

10 Q: Did the marketing team consult legal before implementing this strategy?

20 Q: Were there any objections raised during these consultations?

12 Q: "Do you recall the bonus payout structure described in the document?"

25 Q: "Was this structure intended to incentivize OxyContin sales over MS Contin?"

10 Q: "Did you personally approve this bonus structure?"

25 Q: "Were you aware of any risks associated with this strategy?"

15 Q: "Exhibit 45 references 'dependency concerns in Kentucky.' Were you aware of this?"

25 Q: "Was there a policy to address addiction risks?"

12 Q: "Exhibit 46 mentions a 2001 internal memo. Do you recognize it?"

25 Q: "The memo highlights 'targeting high-prescribing physicians.' Was this a focus?"

15 Q: "Did the strategy include monitoring prescription data?"

25 Q: "Were there safeguards to prevent misuse of this data?"

15 | Q. "You still saw the advantage of getting OxyContin CR uncontrolled?" A. "Yes, we were exploring options to appeal the German decision."

25 | Q. "Was this related to the Agreed Statement of Facts in 2007?" A. "No, this was prior to that, in 1998."

10 | MR. STRAUBER: "Mark this as Exhibit 9." THE WITNESS: "You can keep it." Q. "Did you review this document before today?" A. "Yes, briefly."

20 | Q. "Does this document reflect your understanding at the time?" A. "It aligns with our strategy discussions in 1998."

25 | Q. "Was there any improper conduct in pursuing this strategy?" A. "No, it was within regulatory guidelines."

10 | Q. "Did you consult external counsel regarding the German decision?" A. "Yes, we sought advice on potential appeals."

20 | Q. "Was the appeal ultimately filed?" A. "No, we decided against it after further review."

25 | Q. "Why was the appeal abandoned?" A. "The likelihood of success was deemed too low."

10 | Q. "Did this decision impact your market strategy?" A. "Yes, we adjusted our approach in Europe accordingly."

20 | Q. "Were there financial implications?" A. "Yes, it affected projected revenues for that region."

25 | Q. "Did you communicate this internally?" A. "Yes, through a series of memos and meetings."

10 | Q. "Were these communications documented?" A. "Yes, they are part of our internal records."

20 | Q. "Did you personally draft any of these memos?" A. "I contributed to some, but not all."

25 | Q. "Were these memos shared externally?" A. "No, they were strictly internal documents."

10 | Q. "Did you face any challenges implementing the adjusted strategy?" A. "Yes, there were logistical and regulatory hurdles."

20 | Q. "How were these hurdles addressed?" A. "Through collaboration with local teams and external consultants."

25 | Q. "Did this require additional resources?" A. "Yes, both financial and personnel investments."

10 | Q. "Were these investments approved by senior management?" A. "Yes, after thorough review and justification."

20 | Q. "Did this strategy ultimately succeed?" A. "It had mixed results; some goals were achieved, others were not."

25 | Q. "Were lessons learned from this experience?" A. "Yes, it informed future market entry strategies."

11 | Q: "OxyContin...attention to our newest product, OxyContin Tablets (controlled-release Oxycodone HCl) and have included some literature. Most important to your practice, Did I read that correctly?" A: "Yes."

